

SmartCare Multispeciality Hospital — Patient Admission & Discharge Guide

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1. Purpose & Scope

1.1 Purpose

This operational guideline delineates the administrative and clinical procedures governing patient admission, bed allocation, inpatient stay protocols, room category management, multidisciplinary care coordination, discharge planning, pharmacy reconciliation, and post-discharge continuity of care at SmartCare Multispeciality Hospital.

1.2 Scope

This policy applies to all planned inpatient admissions, day-care surgical cases, direct emergency admissions, step-down transfers, and discharge processes across all clinical divisions and intensive care units.

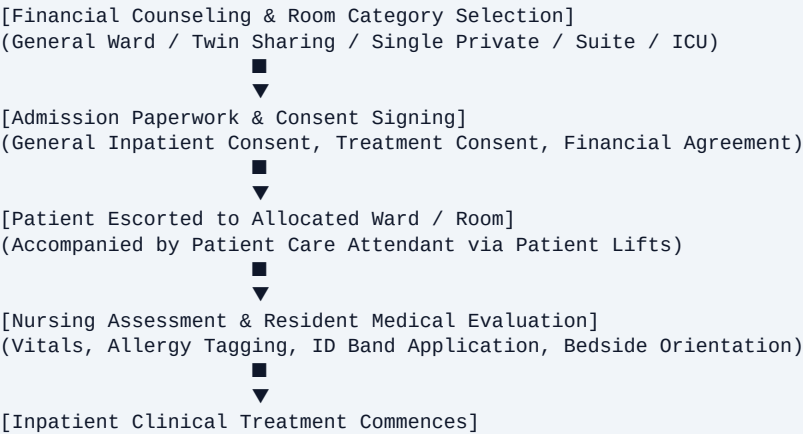
2. Inpatient Admission Classifications

SmartCare Hospital categorizes hospitalizations into three distinct clinical pathways:

- Planned (Elective) Admission:**
 - Initiated by an outpatient consultant for scheduled medical therapies, diagnostic workups, or elective surgical operations.
 - Admission is booked at least 24 to 72 hours in advance through the Central Bed Management Desk.
- Emergency (Unplanned) Admission:**
 - Initiated through the 24/7 Emergency & Trauma Center (Gate 1) when a patient requires immediate resuscitation, intensive care, or urgent surgical intervention.
 - Bed allocation is expedited by the Emergency Nursing Coordinator.
- Day-Care Admission:**
 - Short-stay clinical admissions (under 12 to 24 hours) for minor surgical operations, endoscopies, dialysis sessions, or chemotherapy infusions where overnight stay is not medically required.

3. End-to-End Admission Workflow





4. Required Documents for Inpatient Admission

Patients or attendants must present the following documents at the **Inpatient Registration Desk**:

- 1. **Treating Doctor's Signed Admission Advice Slip** (specifying provisional diagnosis, planned procedure, and anticipated length of stay).
- 2. **Government-Issued Photo ID of the Patient** (Aadhaar Card, Passport, Voter ID, or Driving License).
- 3. **Government-Issued Photo ID of Primary Attendant / Next-of-Kin**.
- 4. **SmartCare UHID Card** (or previous hospital registration slips).
- 5. **Complete Past Medical Dossier** (past discharge summaries, ongoing prescriptions, recent ECG, blood reports, and diagnostic imaging).
- 6. **Health Insurance E-Card & Pre-Auth Documents** (if seeking cashless hospitalization).
- 7. **Emergency Contact Numbers** (minimum two verified contact numbers).

5. Inpatient Room Categories & Amenities

SmartCare Hospital offers diverse accommodation categories tailored to clinical requirements and patient comfort:

Room Category	Clinical & Living Amenities	Attendant Accommodation	Visitor Entitlement
General Ward (Multi-Bed)	6-bed partitioned bays, central air conditioning, nurse call bell, shared attached restroom.	1 bedside chair for attendant.	1 visitor at a time during evening visiting hours.
Semi-Private (Twin Sharing)	2 motorized beds per room, privacy curtain, shared attached restroom, LCD TV, bedside storage.	1 reclining attendant couch.	1 visitor at a time during visiting hours.
Single Private Deluxe	Private air-conditioned room, attached bathroom, motorized patient bed, refrigerator, LED TV, Wi-Fi, telephone.	1 dedicated sofa-cum-bed with bedding.	2 visitors at a time during visiting hours.
Executive Suite	Patient suite + separate furnished living room, two bathrooms, microwave, dining counter, luxury amenities.	Dedicated attendant suite and sleeping couch.	Flexible visiting hours (subject to patient rest).

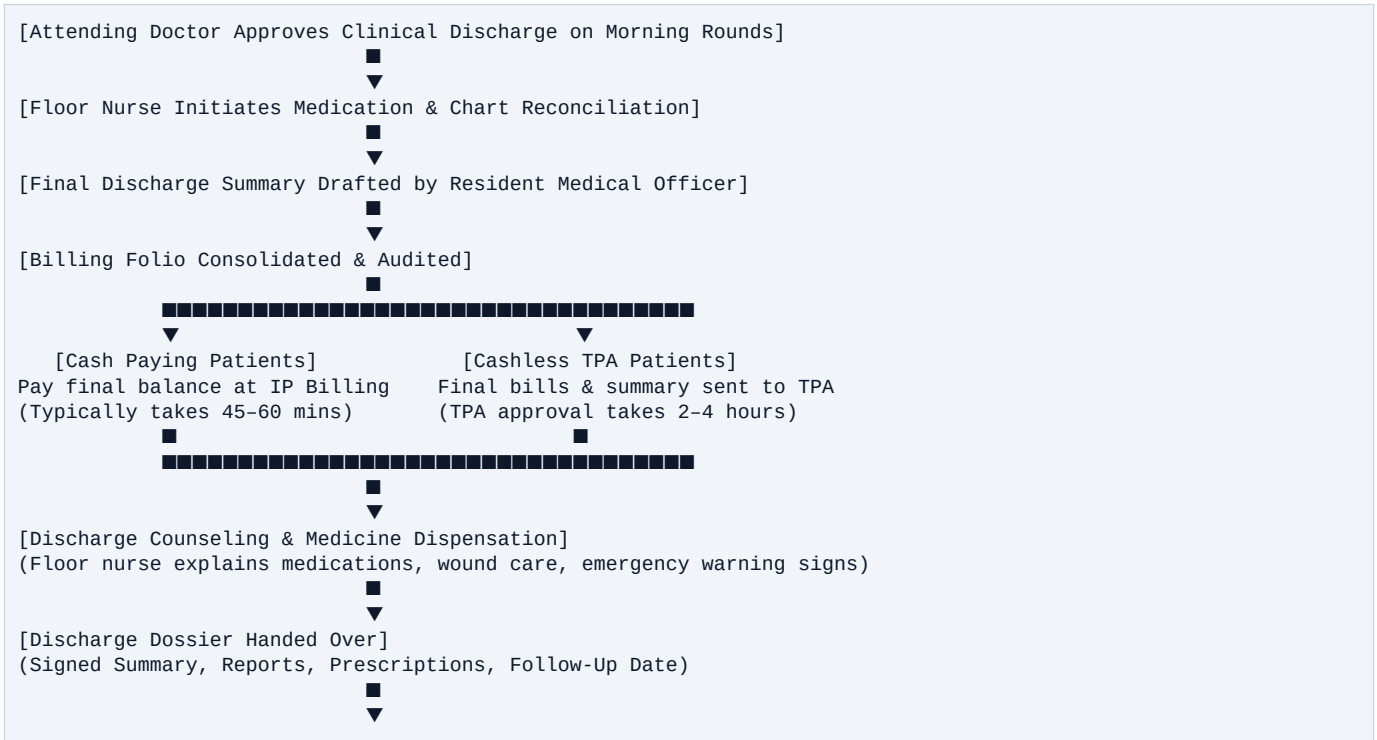
Room Category	Clinical & Living Amenities	Attendant Accommodation	Visitor Entitlement
Intensive Care Units (ICU/CCU/NICU)	Continuous life-support monitoring, ventilator provision, high nurse staffing, strict sterile isolation.	Attendants wait in dedicated ICU Family Lounge.	1 attendant for 30 minutes twice daily.

6. Hospital Guidelines During Inpatient Stay

- Patient Identification Wristband:** Every admitted patient must wear their color-coded hospital ID band (White = Standard; Red = Known Allergy; Yellow = Fall Risk) at all times until discharge.
- Attendant Passes:** One primary 24-hour Attendant Pass and one Visitor Pass are issued per patient. Passes must be visibly worn when moving through the campus.
- Consultant Clinical Rounds:** Primary attending consultants conduct daily inpatient rounds between **8:30 AM – 11:00 AM** and evening rounds between **5:00 PM – 7:30 PM**.
- Hospital Meals & Clinical Nutrition:** Customized therapeutic meals planned by clinical dietitians are served directly to the patient's room:
 - Breakfast: 7:30 AM – 8:30 AM
 - Lunch: 12:30 PM – 1:30 PM
 - Evening Snack: 4:30 PM – 5:00 PM
 - Dinner: 7:30 PM – 8:30 PM
- Note: Outside food is strictly prohibited in ICUs and surgical wards.*
- Visiting Hours & Regulations:**
 - Inpatient Wards: Daily **4:30 PM – 7:00 PM**; Sundays additional morning slot: **10:30 AM – 11:30 AM**.
 - ICUs: Strictly **11:00 AM – 11:30 AM** and **5:00 PM – 5:30 PM** (one attendant only).
 - Children under 12 years of age are not permitted in inpatient clinical areas to prevent infection transmission.

7. Comprehensive Discharge Process

Discharge planning begins at the time of admission to ensure a safe, comfortable, and seamless transition home:



[Discharge Gate Pass Issued ■■■ Patient Vacates Room]

7.1 Key Discharge Documents Provided:

1. **Official Medical Discharge Summary:** Detailing clinical admission diagnosis, surgical operative notes, hospital course, diagnostic summaries, condition at discharge, and vital signs.
2. **Discharge Medication Prescription:** Clear itemized list of take-home drugs, exact dosage, timing, duration, and food relationship.
3. **Follow-Up Appointment Slip:** Scheduled date, time, and room number for post-discharge review with the primary consultant.
4. **Emergency Warning Signs Checklist:** Red-flag symptoms (e.g., fever >101°F, bleeding, sudden pain, wound discharge) warranting immediate return to the 24/7 emergency room.
5. **Settled Final Hospital Bill & Payment Receipts.**

8. RAG Knowledge Search & Question Triggers

- **Trigger Queries:** "What time is discharge?", "How long does insurance discharge take?", "What documents do I need for hospital admission?", "What are ICU visiting hours?", "Can children visit admitted patients?", "What is included in a private deluxe room?", "Can I bring home food for a patient?", "What is an Against Medical Advice (LAMA) discharge?"
- **Standardized Fact Base:** Inpatient registration is on Level 2. Wards visiting hours: 4:30 PM – 7:00 PM daily. ICU visiting hours: 11:00 AM – 11:30 AM and 5:00 PM – 5:30 PM. Children <12 not permitted in wards. Cash discharge takes ~60 mins; insurance discharge takes 2–4 hours for TPA clearance.

9. Frequently Asked Questions (FAQs)

Q1: What is the check-in time for planned surgical admissions?

A: Planned surgical patients are typically requested to report to the Inpatient Admission Desk between **6:00 AM and 7:00 AM** on the morning of surgery, or the evening prior, as specified by the surgeon and anesthesiologist.

Q2: What personal items should I pack for an inpatient stay?

A: Bring comfortable loose-fitting clothing, non-slip slippers, personal toiletries (toothbrush, comb), current medications in original packaging, and previous medical records. Do not bring large amounts of cash or jewelry.

Q3: Can an attendant stay overnight in the room with the patient?

A: Yes. One healthy adult attendant is permitted to stay overnight in Semi-Private rooms, Single Private Deluxe rooms, and Executive Suites. Overnight attendants are not permitted inside ICU wards.

Q4: What are the visiting hours for admitted patients?

A: For general and private rooms, visiting hours are daily from **4:30 PM to 7:00 PM**, with an additional morning slot on Sundays from **10:30 AM to 11:30 AM**.

Q5: Why are children under 12 not allowed in inpatient wards?

A: Young children have developing immune systems and are at higher risk of acquiring hospital-associated infections, and may also inadvertently transmit childhood viral infections to vulnerable recovering patients.

Q6: How long does the discharge process take?

A: For cash-paying patients, discharge billing and pharmacy reconciliation take approximately **1 to 2 hours**. For cashless insurance patients, the process takes **2 to 4 hours** due to mandatory final medical audits by external insurance TPAs.

Q7: What happens if a patient wants to leave the hospital against medical advice (LAMA / DAMA)?

A: If a patient or legal guardian insists on discharge despite clinical warnings from the treating doctor, they must sign a "Leave Against Medical Advice" (LAMA) document acknowledging that they assume full legal and health responsibility for potential clinical deterioration.

Q8: Can I request an ambulance to take me home after discharge?

A: Yes. Non-emergency patient transport ambulances can be booked through the Floor Nursing Supervisor or by contacting Central Ambulance Services at +91-40-6999-0000. Standard transport tariffs apply.

Q9: What should I do if I feel feverish or experience wound bleeding after reaching home?

A: Review your emergency warning signs sheet. If experiencing acute fever, bleeding, severe shortness of breath, or uncontrollable pain, do not wait for an OPD visit—proceed immediately to the 24/7 SmartCare Emergency Center at Gate 1.

Q10: Are laundry and linen services provided for attendants?

A: Hospital gowns and clean bed linens are provided and laundered daily for patients. Attendant bed linen is provided in Private and Suite rooms. Commercial laundry for personal clothing is available upon request for long-stay patients.

Q11: Can an attendant get meals from the hospital?

A: Yes. Attendants may purchase meals at the Ground Floor Cafeteria or order attendant meal trays delivered to the room by contacting the Dietary Call Center (Extension 1055).

Q12: Can I choose my room category?

A: Yes, room category is chosen at the time of admission based on your personal preference, insurance entitlement, and live bed availability.

Q13: What happens if the room category I requested is full?

A: You will be temporarily accommodated in an alternate available room category and transferred to your preferred category as soon as a bed becomes sanitized and available.

Q14: Are patients allowed to leave their room to walk in the corridors?

A: Medically stable patients may walk in designated floor corridors under nursing supervision. Post-operative and fall-risk patients must always be accompanied by an attendant or nursing orderly.

Q15: What is a "Day-Care" admission?

A: A day-care admission is a planned hospital stay lasting between 3 and 12 hours for minor procedures (e.g., endoscopy, cataract surgery, chemotherapy, minor biopsies) where overnight bed stay is unnecessary.

Q16: Can I get my discharge summary emailed to me?

A: Yes. Digital PDF copies of your signed discharge summary and all diagnostic reports are automatically available on the SmartCare Patient Portal and can be sent to your registered email.

Q17: Is smoking or alcohol permitted in the hospital rooms?

A: Absolutely not. SmartCare Hospital is a 100% smoke-free, tobacco-free, and alcohol-free healthcare campus. Violations will result in immediate security intervention and statutory fines.

Q18: What is the discharge cut-off time to avoid extra room charges?

A: Patients are expected to vacate the room within **1 hour** of receiving their final Discharge Gate Pass. If vacating extends beyond 2 hours after final clearance, a partial day room charge may apply.

Q19: How are patient medications managed during the hospital stay?

A: All medications administered during your stay are stored in the floor medication room and administered strictly by licensed registered nurses in accordance with doctor orders to prevent medication errors.

Q20: Can I keep my home medications with me in the hospital room?

A: No. To prevent accidental double-dosing or adverse drug interactions, all personal medications brought from home must be declared to the floor nursing team upon admission.

Q21: Does SmartCare have a secure locker for valuables in private rooms?

A: Single Private and Executive Suite rooms feature digital electronic safes. However, patients are strongly advised to send expensive jewelry and large sums of cash home with family.

Q22: Who assists senior citizens and disabled patients during discharge?

A: Dedicated patient care coordinators and wheelchair orderlies will assist the patient from their bedside to their waiting vehicle at the main porch.

Q23: When do I come back to see the doctor after discharge?

A: Your follow-up review date is specified on your discharge summary (typically between 5 and 7 days post-discharge). This follow-up visit is complimentary with your treating consultant.

Q24: What happens if my doctor orders an urgent transfer to the ICU during my ward stay?

A: If vital parameters deteriorate, the Rapid Response Team (RRT) will immediately transfer the patient to the intensive care unit. The attendant will be briefed by the primary consultant regarding the clinical rationale.

Q25: Can I request a copy of my nursing notes and temperature charts?

A: Yes. You can request copies of complete indoor case records, nursing charts, and doctor orders through the Medical Records Department (MRD) by submitting a formal records request.

10. Official Contact Directory

- **Inpatient Admission & Bed Management Desk:** Level 2 Central Atrium
- **Direct Telephone:** +91-40-6999-1155 / +91-40-6999-1156
- **Discharge Care Coordination Cell:** Level 2 Desk 4, Ext. 2044
- **Email:** admissions@smartcarehospital-demo.org / dischargecare@smartcarehospital-demo.org
- **Operating Hours:** Inpatient Registration & Emergency Admissions are open **24/7**.